Kidney-Protective Medications

Synthetic reference for demo purposes — general guidance, not a clinical guideline or St. Luke's policy.

SGLT2 inhibitors (e.g., empagliflozin, dapagliflozin)

Slow CKD progression and reduce cardiovascular and heart-failure risk. Considered for CKD patients with albuminuria (with or without type 2 diabetes) when eGFR is above the label threshold. Counsel on sick-day rules and volume status.

GLP-1 receptor agonists (e.g., semaglutide)

Provide cardiovascular benefit and support weight and glycemic management; increasingly used in CKD with type 2 diabetes. Considered kidney-supportive as part of a broader regimen.

ACE inhibitors / ARBs

First-line for CKD with albuminuria and/or hypertension; reduce proteinuria and slow progression. Monitor potassium and creatinine after initiation or dose change.

Care-gap note

Patients with advanced CKD (G3b–G5) who are **not on any kidney-protective therapy** represent a treatment gap worth review — especially those with albuminuria.